ADR REPORTING & DOCUMENTATION

Pharmacovigilance Internship Task

Field	Details
Submitted by	Atharva Dhage
Course	Pharm D
Task	ADR Reporting & Documentation
Objective	Complete a structured ADR reporting form based on a case scenario
Submission format	Completed ADR Reporting Form — PDF

Case-data note

Educational example prepared from the information available in the current task context. It is not a real patient record. If the Workora Reporting Case Pack contains different fictional case details, replace this example with the exact case-pack information before final submission.

1. Patient Information

Field	Completed information
Patient identifier / initials	PT-001
Age	25 years
Sex	Female
Weight	Unknown / not provided
Relevant medical history	No significant comorbidity reported

2. Adverse Reaction Details

Field	Completed information
Reaction / event	Generalized erythematous skin rash with itching (pruritus)
Reaction start	Day 2 after starting amoxicillin
Reaction stop / resolution	Substantially improved by Day 4
Outcome	Recovered / improving after withdrawal
Seriousness	Not reported as serious in the educational scenario
Severity	Mild to moderate; exact grade not provided
Management	Suspected medicine discontinued; observation/supportive care

3. Suspect Product / Drug Details

Field	Completed information
Suspect drug	Amoxicillin
Strength	500 mg
Dose / frequency	500 mg orally, three times daily
Route	Oral
Indication	Bacterial infection
Therapy start	Day 0
Therapy stop	Day 2
Action taken	Amoxicillin discontinued
Dechallenge	Positive — rash substantially improved after withdrawal
Rechallenge	Not performed / not reported

4. Concomitant / Other Medicines

Medicine	Use / timing	Assessment
Paracetamol 500 mg	As needed for fever	Concomitant medicine; no information linking it to the rash

5. Clinical Narrative

Case narrative: A 25-year-old female with no significant reported comorbidity was started on amoxicillin 500 mg orally three times daily for a bacterial infection. Paracetamol 500 mg was used as needed for fever. On Day 2 after initiation of amoxicillin, the patient developed a generalized erythematous skin rash with itching. Amoxicillin was discontinued on Day 2. The rash substantially improved by Day 4. No rechallenge was reported. Based on the available information, amoxicillin is considered the suspect medicine and the event is documented as a suspected ADR.

6. Relevant Investigations / History

Item	Information
Laboratory / diagnostic findings	Not provided
Allergy history	Not provided
Other relevant medical history	No significant comorbidity reported
Prior medication history	Not provided

7. ADR Reporting Assessment

Assessment element	Finding
Identifiable patient	Yes — PT-001
Suspect drug	Yes — amoxicillin
Adverse reaction	Yes — generalized rash with pruritus
Identifiable reporter	Atharva Dhage — Student / Pharmacovigilance Intern
Temporal relationship	Compatible: reaction occurred 2 days after initiation
Dechallenge	Positive
Rechallenge	Not performed / not reported
Causality	Suspected drug-related reaction; formal causality assessment required
Case status	Educational pharmacovigilance case documentation

8. Documentation Checklist

Documentation item	Status
Patient identifier and demographics	Completed
Reaction description and onset	Completed
Suspect drug, dose, route and indication	Completed
Therapy start/stop information	Completed
Concomitant medicine	Completed
Relevant history / investigations	Recorded or marked unknown
Outcome and management	Completed
Dechallenge / rechallenge	Documented
Reporter details	Student/intern identifier recorded; contact details

9. Conclusion

The case has been documented in a structured ADR-reporting format. The key elements include an identifiable patient, suspect medicine, adverse event, clinical timeline, outcome, concomitant medication, and reporter identification. The timeline and positive dechallenge support suspicion of an amoxicillin-associated skin reaction. Formal causality, seriousness, severity and regulatory processing should be completed using the applicable workflow and all available case information.

References

1. U.S. Food and Drug Administration. Instructions for Completing Form FDA 3500 / MedWatch. 2. Indian Pharmacopoeia Commission. Pharmacovigilance Programme of India (PvPI). 3. ICH E2D. Post-Approval Safety Data Management: Definitions and Standards for Individual Case Safety Reports.

Educational use only. No real patient identifiers are included.